

Public Health Emergency Operations Center "COUSP DRC"

MVE-17 Incident Management System

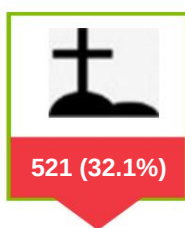
Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°052/MVB_05/07/2026

Affected provinces (3)	ITURI, NORTH KIVU & SOUTH KIVU
Affected Health Zones (36)	- Ituri (24/36): Aru, Aungba, Bambu, Bunia, Damas, Drodro, Gety, Kilo, Komanda, Lita, Logo, Mambasa, Mangala, Mongbwalu, Nizi, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Lolwa. - North Kivu (11/34): Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako and Musienene. - South Kivu (1/34): Miti-Murhesa
Reporting date	July 5, 2026
Publication date	July 6, 2026



Total confirmed cases
for the 3 provinces



Cumulative deaths
among confirmed cases
and case fatality rate



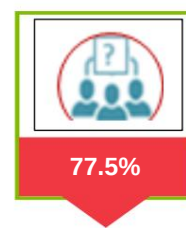
Patients in
isolation -
hospitalization



Cumulative number
of recoveries



Suspected cases of
the day



Contact tracing rates
for the 2 provinces

Suspicious deaths occurring in the community are subject to investigation.

further investigation with a view to possible reclassification (Non-cases, probable case)

* Data updated following the DHIS2 database cleaning and harmonization process, including the consolidation of information from health zones, laboratories and case management structures.

1. HIGHLIGHTS

Sixty -three (63) new confirmed cases, including 8 community deaths, were reported during the day on July 5, 2026, in the provinces of Ituri (60 cases, including 8 deaths) and North Kivu (3 cases). These confirmed cases come from the health zones of **Bunia (14), Mongbwalu (10), Nia-Nia (8), Bambu (7), Mangala (7), Nizi (6), Rwampara (5), Fataki (1), Rimba (1), Nyankunde (1)** in Ituri and **Musienene (2) and Masereka (1)** in North Kivu.

- **Seven (7)** deaths were recorded among patients with Ebola virus disease in Ebola treatment centers in the Ituri provinces (2 at Ebola Treatment Center Nyankunde, 2 at Ebola Treatment Center Mongbwalu, 1 at Ebola Treatment Center CME, 1 at Ebola Treatment Center Bunia and 1 at Ebola Treatment Center Bambu).
- **Twenty (20)** patients with Ebola virus disease in Ituri province (4 at CTE CME, 4 at CTE Elikya, 3 at CTE Mongbwalu, 3 at CTE Nyankunde, 3 at CT Sota, 2 at CTE Rwampara and 1 at CT Bambu) have been declared cured after obtaining negative control tests.
- Cessation of response activities in the Bunia health zone by service providers.

2. EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Ituri province, located in the northeast of the Democratic Republic of Congo, shares a long and porous border with Uganda and South Sudan. For over two decades, it has faced a chronic humanitarian crisis linked to armed conflict and recurring population displacements. Its population is estimated at over 8 million, including more than one million internally displaced persons. The Mongbwalu health zone, considered the starting point of the epidemic, is located in the Djugu territory, 70 km from Bunia. This area is characterized by the presence of armed groups, frequent population movements towards Uganda, and numerous artisanal mining sites attracting migrant workers. In September 2018, Ituri province had already been affected by the 10th Ebola Virus Disease (EVD) epidemic.

In North Kivu, the current epidemic, caused by the Bundibugyo ebolavirus, was officially declared on May 15, 2026, by the Minister of Public Health. The three affected provinces have a total population estimated at nearly 15 million inhabitants, with massive internal displacement and cross-border flows to neighboring countries.

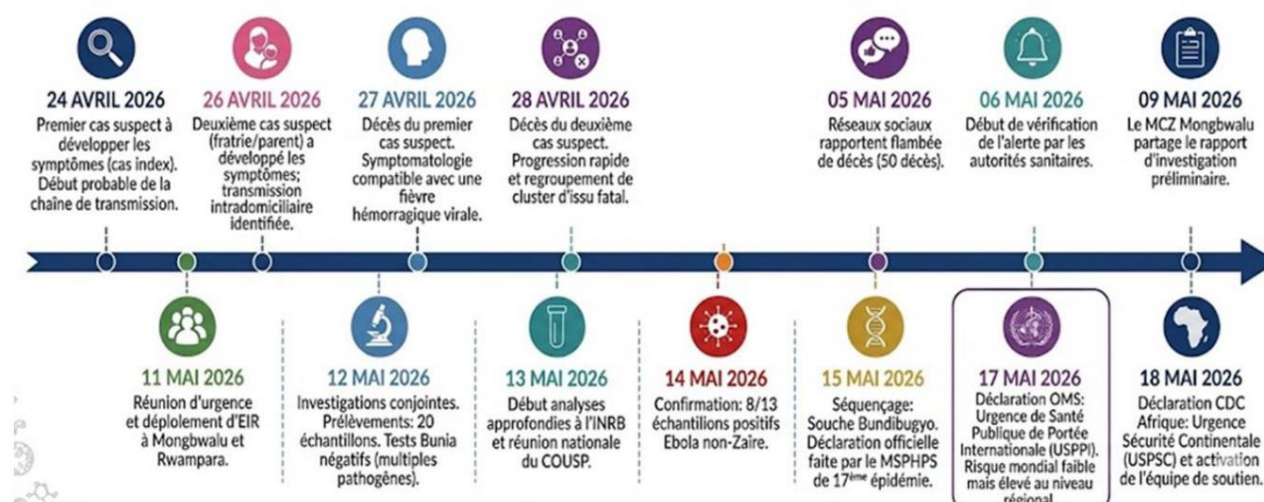


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic.

3. DETAILED EPIDEMIOLOGICAL ANALYSIS

3.1 Spatial distribution and active hotspots as of July 5, 2026

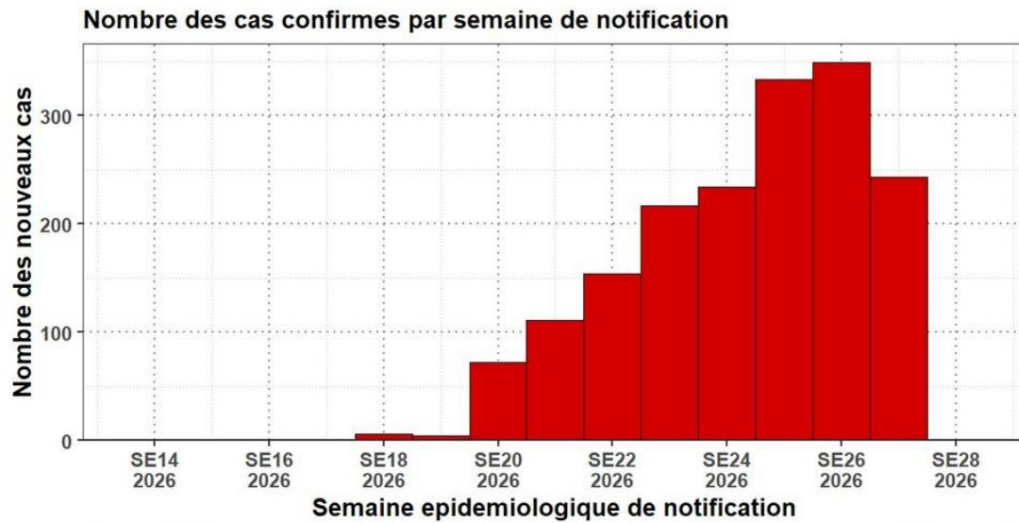
Table 1. Distribution of confirmed cases and deaths by affected province as of July 5, 2026.

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	1477	439*	29.7%	24 out of 36 (66.7%)	60
North Kivu	144	81	56.3%	11 out of 34 (32.3%)	3
South Kivu	3	1	33.3%	1 out of 34 (2.9%)	0
Total	1,624	521	32.1%	36 out of 104 (34.6%)	63

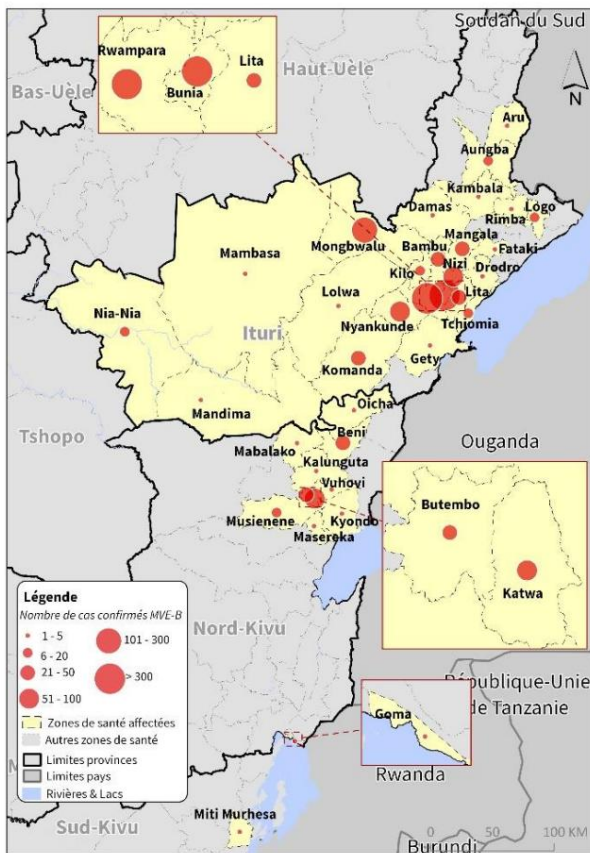
* 7 deaths in 211 patients were recorded among patients with EVD (previous confirmed cases) in Ebola Treatment Centers.

At the end of the day on July 5, 2026, no new health zones were affected. The number of affected health zones remains at 36/104 (section 3.1). Since May 26, 2026, South Kivu province has not recorded any confirmed cases, suggesting an absence of recent transmission.

3.2 Time-bound analysis: An increasing number of confirmed cases is observed from one week to the next, reflecting ongoing community transmission of the disease, although the number of cases appeared to be stable during epidemiological weeks 23 and 24. The intensification of public health actions in epidemiological and biological surveillance (decentralization of diagnostic capabilities) contributed to early detection, confirming continued and increased community transmission in week 25. Compared to all the weeks in which cases have been recorded since the beginning of the epidemic, weeks 25 and 26 represent the peak so far, with more than 300 cases per week.



3.3 Epidemic mapping



Figures 2 and 3. Spatial distribution of confirmed Ebola cases by affected health zones and in the three affected provinces as of July 5, 2026.

Table 2. Distribution of confirmed cases and deaths of Ebola virus disease by province and health zone (July 5, 2026)

Province / Health Zone	Cumulative confirmed cases (n)	Cumulative confirmed deaths (n)	Case fatality rate (CFR%)
ITURI	1477	439	29.7%
Bunia	452	121	26.9
Rwampara	349	72	20.6
Mongbwalu	288	149	51.7
Nyankunde	96	19	19.8
Nizi	78	19	24.4
Lita	33	12	36.4
Komanda	22	8	36.4
Bamboo	25	6	24.0
Kilo	9	1	11.1
Mangala	33	14	42.4
Tchomia	15	2	13.3
Damascus	5	0	0.0
Aungba	6	2	33.3
Rimba	4	0	0.0
Aru	3		33.3
Mambasa	2		50.0
Logo	7	1	0.0
Drodro	3	1	100.0
Gety		0	0.0
Kambala	1	3	100.0
Nia-Nia	2	0	25.0
Fataki	20 2	2	0.0
Mandima	4		25.0
Lolwa			100.0
Other areas not yet identified	1 17	50 110	0.0
NORTH KIVU	144	81	56.3
Butembo	40	18	45.0
Katwa	52	38	73.1
Blessed	29	17	58.6
Oicha	3	2	66.7
Kalunguta	2		50.0
Kyondo	4	1	50.0
Goma		2	0.0
Masereka	1	0	0.0
Vuhovi			100.0
Mabalako		0	0.0
Musienene	3	1	25.0
SOUTH KIVU	1 18 3	0 21	33.3
Miti-Murhesa	3	1	33.3
TOTAL	1,624	521	32.1%

In the past 24 hours, **63 new confirmed cases and 15 deaths have been recorded** in the provinces of Ituri (60 cases and 15 deaths) and North Kivu (3 cases), highlighting the progression of the epidemiological situation of the 17th Ebola Virus Disease (EVD) epidemic in the Democratic Republic of Congo. Thus, the cumulative total stands at 1,624 confirmed cases and 521 deaths, expressing an overall case fatality rate of 32.1%.

Provincial dynamics and case concentration

The **Ituri** province alone accounts for **90.9%** of cases (n=1,477) and **84.3%** of deaths (n=439), representing a case fatality rate of **29.7%**, making it the absolute epicenter of the epidemic. The health zones of Bunia (452 cases), Rwampara (349 cases), Mongbwalu (288 cases), Nyankunde (96 cases), and Nizi (78) remain the most affected, accounting for approximately 85.5% of reported cases at the provincial level and 77.8% at the national level. Within these same health zones, moderate case fatality rates were observed in the Rwampara (20.6%) and Bunia (26.9%) health zones, while the **Mongbwalu** zone also showed a high case fatality rate of **51.7%** (149 deaths out of 288 cases), suggesting persistent challenges in early intervention or access to care. It should be noted that 17 additional cases (with no associated deaths) were reported, while the health zones of origin remain to be identified, which could indicate a geographical spread requiring further investigation.

Severity of the Epidemic in North Kivu: A total of 144 confirmed cases and 81 deaths have been reported in North Kivu province since the start of the epidemic, representing a **case fatality rate of 56.3%**. This province presents the most concerning profile in terms of severity, with a case fatality rate significantly higher than the national average. This high case fatality rate is concentrated in the health zones of **Katwa** (73.1%), **Oicha** (66.7%), **Beni** (58.6%), **Kalunguta**, and **Kyondo** (50.0%), respectively. The Katwa health zone has the highest number of reported cases (n=52), followed by the Butembo health zone (n=40) with a case fatality rate of 45.0%. A case fatality rate of 100% was exceptionally observed in the Vuhovi health zone (which has a small population).

4. RESPONSE ACTIONS BY PILLAR

4.1 Coordination

National / Ituri & Kinshasa

- Holding the coordination meeting with the response team and technical partners and financial support for field activities.

National level :

- Coordination meeting with an agenda: 100%
- Action items completed for daily coordination meetings: 80% **Provincial level :**

- Coordination meeting with an agenda: 100% for ITURI and North Kivu; South Kivu: ND

- Action items completed for daily coordination meetings: N/A for the three provinces (ITURI, North Kivu and South Kivu)

Zonal Level : •

Coordination meeting with an agenda: N/A

- Action items completed for daily coordination meetings: N/A

4.2 Epidemiological surveillance

Table 3. Management of epidemiological alerts (24h) as of July 5, 2026

Indicator	Ituri	North Kivu	South Kivu	Total
Report (D-1)			4	26
New Alerts		0	11	901
Total alerts for today		662	15	927
Alerts investigated		662	15	824
Investigation rate		662	100.0%	88.9%
Alerts confirmed	Alive		5	112
	Deceased	22,228,250,147 58.8% 6100.0% 45 19	0	23
Total suspected cases for today	66	64	5	135

During the day of July 5, 2026, a total of **927 alerts** were recorded, including 901 new alerts, demonstrating the continued active epidemiological surveillance in the affected provinces.

The majority of alerts were reported in North Kivu (**662, 71.4%**), followed by Ituri (**250, 26.9%**) and South Kivu (**15%**). (**1.6%**).

Of the alerts recorded that day, 824 (88.9%) were investigated. The investigation rate reached 100% in North Kivu and South Kivu, while it stood at **58.8%** in Ituri, highlighting a need to strengthen investigative capacities in that province.

The investigations led to the confirmation of 135 suspected cases, of which 112 (83.0%) involved living individuals and **23 (17.0%)** were community deaths. Ituri recorded the highest number of suspected cases.

validated **66 (48.9%)**, followed by North Kivu **64 (47.4%)** and South Kivu **5 (3.7%)**.

Continued strengthening of surveillance, rapid investigation of alerts and early detection of cases, particularly in Ituri, remains essential to interrupt the chains of transmission.

Table 4. Follow-up of contacts of confirmed cases (1) as of July 5, 2026

Province	Contacts under follow-up (n)	Contacts viewed (n)	Follow-up rate (%)
Ituri*	9,757	7,574	77.6%
North Kivu	2,655	2,050	77.2%
Total	12,412	9,624	77.5%

As of July 5th, **12,412** contacts were under surveillance in the provinces of Ituri and North Kivu. Of these, 9,624 (77.5%) were actually seen, corresponding to an overall follow-up rate of 77.5%.

Ituri had the highest number of contacts under follow-up, 9,757 (78.6% of the total), with 7,574 contacts seen, representing a follow-up rate of 77.6%. In North Kivu, 2,655 contacts were under follow-up, of which 2,050 (77.2%) were visited.

Contact tracing performance is comparable between the two provinces; however, the overall follow-up rate remains below the recommended operational threshold for optimal contact monitoring. Strengthening daily contact tracing, particularly of those who have not been seen, remains a priority to ensure the early detection of secondary cases and limit the risk of community transmission.

Table 5. Contact tracing of confirmed cases (2) as of July 5, 2026

Province	Number of New Contacts		Number of contacts Became a Suspect		Number of contacts made in the last 21 days	
	n		n		n	%
Ituri	538	88.1 %	5	62.5 %	52	36.1%
North Kivu	73	11.9%	3	37.5%	92	63.9%
Total	611	100.0%	8	100.0%	144	100.0%

The day ended on July 5th with **611 new contacts** listed, the majority in Ituri (538, 88.1%), while North Kivu recorded 73 (11.9%). Among the contacts monitored, eight (8) developed symptoms consistent with the definition of a suspected Ebola case, five (62.5%) in Ituri and three (37.5%) in North Kivu, requiring immediate investigation and management in accordance with current procedures.

In addition, 144 contacts successfully completed their 21-day follow-up period without developing symptoms consistent with the disease, including 92 (63.9%) in North Kivu and **52 (36.1%)** in Ituri.

These results demonstrate the continuation of contact tracing and follow-up activities, while highlighting the need to maintain rigorous surveillance to ensure early detection of secondary cases and to quickly interrupt any potential chain of transmission.

Update on checkpoint and point of entry (PoC/PoE) activities

- **Ituri** : Eight (8) active alerts were notified to the PoE/PoC, of which six (6) were validated. Five (5) validated alerts were referred to the CT/CTE, and the transfer of one alert was in progress at the close of the day.
One alert was invalidated after re-sampling, and another left the PoC before the investigation team arrived. The alerts were reported to the PoCs at Base Yedi, BEY, Foner Mambasa, Anzida, and Mudzibala.
- **North Kivu**: No alerts have been notified to PoE/PoC;

Table 6. Monitoring of indicators at Points of Entry and Points of Control (PoE/PoC), as of July 5, 2026

INDICATORS	Ituri	North Kivu	Overall
Proportion of PoC enabled and PoE enhanced	98.1% (54/55)	100.0% (14/14)	98.6% (68/69)
Number of people who went through PoE/PoC testing,	85,218	50,644	135,862
% of travelers screened, % of	94.6%	97.2%	95.8%
travelers who washed their hands, % of	95.0%	97.2%	95.8%
travelers made aware of the issue	99.7%	97.2%	98.8%
Number of alerts notified	8	0	8
Number of validated live alerts		0	6
Number of lifeless bodies intercepted today	60	0	0
Number of problematic contacts intercepted today; % of			0
validated live alerts referred to CT/CTE; % of deceased	0 83.3%	0 0.0%	83.3%
bodies swabed	NA	NA	N / A
Number of positive results of referred suspected cases	0	0	0

4.3. Laboratory

- **Ituri** : 168 samples collected and analyzed (**positivity 35.7%; n=60**).
- **North Kivu** : 82 samples collected and analyzed (**positivity 3.6%; n=3**).

- **South Kivu** : 2 samples collected and currently being analyzed.

4.4. Infection Prevention and Control (IPC)

Ituri: 17 households identified around confirmed cases were decontaminated in the health zones of Rwampara, Komanda, Nyakunde, and Nizi. Two (2) health assessments (HAs) were not carried out due to community resistance and logistical challenges, and 9 failures were reported. The reporting rate was 29.1% (7/24). 16 community health workers (CHWs) were monitored and supported, eight (8) means of transport were decontaminated, 42 local public health workers (LPHs) were trained, and 34 service providers were briefed during formative supervision.

North Kivu: Decontamination at HGR MUSIENENE (the pediatrics room; Internal Medicine woman) and CH SAINT RITA in the AS NDUKO where a confirmed case from the ZS of Butembo had passed; Decontamination of the place of stay of the confirmed case of the HGR referred to the CTE; Of the 27 alerts reported, 27 Swabs , carried out and 22 EDS carried out with 11 reports.

4.5 Holistic care

Seven (7) deaths were recorded among Ebola virus disease (EVD) patients in Ebola treatment centers (ETCs) in Ituri province (1 at the Bunia ETC, 1 at the CME ETC, 2 at the Mongbwalu ETC, 2 at the Nyankunde ETC, and 1 at the Bambu ETC). Three (3) suspected deaths were also reported (1 at the Bunia ETC and 2 at the Mongbwalu ETC).

- **Twenty (20)** patients with Ebola virus disease in Ituri province (2 at CTE Rwampara, 4 at CTE CME, 3 at CTE Mongbwalu, 3 at CTE Nyankunde, 4 at CTE Elikya, 1 at CT Bambu and 3 at CT SOTA) have been declared cured after obtaining negative control tests, including a 3-month-old child at CTE CME.
- By the end of the day on July 5th, 797 hot meals had been served, including 153 to confirmed cases, 132 of the suspected cases, 460 to PPL and 52 to accompanying persons.
- Patient movement :
- Transfer of a confirmed case (PPL) from CT Musienene to CTE Katwa.

Table 7. Patient movement within healthcare facilities (CTE/CT/CI) as of July 5, 2026

Indicator	Ituri	North Kivu	South Kivu	Together
Patients in bed (Day -1)	462	147	21	630
Total admissions (24h)	79	19	6	104
Exits (24h)				
Deceased (Suspects/Confessions)	10	0	0	10
No case	31	17	6	54
Healed	20	0	0	20
Escapees (Suspects/Confessions)	3	1	0	4
Transferred to the HGR	0	0	0	0
Total outflows	64	18	6	88
Patients in isolation (End of Day),	477	148	21	646
including confirmed cases	191	14	0	205
(NC+AC) and suspected cases	286	134	21	441
Overall occupancy rate	89.3%	137.0%	47.7%	94.2%

In total, 646 patients were in isolation, including 205 confirmed cases and 441 suspected cases, for an overall bed occupancy rate of **94.2%**.

Mental health activities and psychosocial support

Table 8. Key indicators of mental health and psychosocial support activities as of July 5, 2026

KEY INDICATORS	ITURI	NORTH KIVU	SOUTH KIVU
New confirmed cases admitted to the CTE who benefited from SMSPS interventions	12 (85.7%)	0	0
Confirmed cases currently being monitored that have received interventions SMSPS	30 (31.2%)	7	0
New suspected cases that benefited from SMSPS interventions	21 (65.6%)	6	1
Suspected cases under follow-up that have received interventions SMSPS	72 (62.0%)	24	2 (100.0%)
Laboratory results have been announced.	23	19	0
Including positive ones	8	0	0
Number of children separated in daycare who benefited from interventions SMSPS	4	0	0
Number of children separated in the community who benefited from SMSPS interventions	0	0	11
Number of accompanying persons at the CTE who benefited from interventions SMSPS	58	21	1
Number of PPLs who benefited from SMSPS interventions	28	9	0
Number of household and community members who benefited from SMSPS interventions	24	0	0
Number of EDS supported	16	0	0

4.6 Continuity of care

- In total, **109 PPLs** are confirmed, including 34 deaths (31.2%) and **46 recoveries** in the province of Ituri.

4.7 Risk communication and community engagement

Ituri: A briefing was held for 380 teachers tasked with raising awareness about Ebola Virus Disease (EVD) in their respective communities. A workshop to redeploy focal points across 15 areas was conducted to improve alert reporting and contact tracing. Awareness messages reached 56,394 people during home visits to 1,009 households in the health zones of Rwampara and Aru. Educational talks reached 46,693 people. Community dialogues engaged 2,780 people, and mass awareness campaigns reached 28,917 people in the health zones of Bunia, Nyakunde, and Aru. Six radio programs and six public service announcements were produced and broadcast. Thirty-six community radio stations were involved, and 15 information, education, and communication (IEC) materials were distributed.

Community feedback: 67 pieces of feedback and informational messages were collected, of which 38 were clarified (56.7%). The main concerns related to the supposed end of the epidemic, the use of thermal scanners, barrier measures in crowded places, and the use of chlorinated water.

Support for the pillars of the response

Surveillance: Support for listing 8 people around a positive case in Shari and 14 people at the Rwampara CTE around 2 suspected cases; identification of addresses for listing, contact tracing and decontamination; listing of 14 contacts around 2 cases; Support for announcing results.

CREC continued awareness-raising in churches, mosques and markets, as well as community activities in the health zones of Rwampara, Aru, Bunia and Nyakunde.

4.8. Logistics

Ituri

- Delivery of IPC supplies to the UNICEF office in Beni (133 kg), to the Beni Health Zone (222 kg) and to the CTE CME (53 kg).
- Receipt of CREC/Surveillance equipment intended for INSP: 200 Samsung Galaxy tablets, 200 tablet cases, 200 charger cases, 1 printer with cartridge, 5 extension cords and 5 ring lights, for a total weight of 224 kg.
- Provision of 25 vehicles and 4 ambulances to the various health pillars and zones to ensure their mobility.
- Supply of operations with 367 litres of fuel.
- Continuation of infrastructure work: construction of a 100-bed CTE in Rwankole; new building of the Rwampara CTE operational; development of the Elikya/Bunia CTE; CME/Bunia, ISTM/Nyakunde, Rwampara, Mongbwalu and Aru CTEs operational.

North Kivu:

- Delivery of PCI inputs, Surveillance and other medical supplies for the M'PA ZS consisting of 30 pairs of protective goggles, 2 liters of hydroalcoholic gel, 15 pieces of liquid soap, 4 cartons of 20 pieces of soap, 5 pieces of metal beds, 5 pieces of padded mattresses.

4.9. Security

- Cessation of response activities in the Bunia health zone by service providers.

4.10. PSEA (Prevention of Sexual Exploitation and Abuse)

Ituri : Briefing activities for stakeholders and community awareness campaigns on the prevention of sexual exploitation and abuse continued in the health zones of Bunia, Rwampara, Mongbwalu, and Aru. No updated figures were reported in the consolidated presentation of July 5, 2026.

5. MAIN CHALLENGES, IMPACTS AND REQUIRED ACTIONS

Challenge	Impact	Actions required
No. 1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, and strengthen team security.
2 Insufficient capacity of CTEs and saturation in North Kivu (137.0%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy additional temporary structures
3 Contact tracking performance not optimal (77.5% vs target 95%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs), enhanced supervision
4 Low reporting of alerts in some affected areas	Undetected transmission, silent spread of the epidemic	Strengthen community surveillance, improve the reporting and rapid investigation of alerts

5	confirmed cases not yet ventilated by health zone (17 cases)	Major gaps in the reconstruction of transmission chains	Conduct a data reconciliation, prune any possible duplicates, and reclassify or clarify the health zone and status (deceased, recovered, active, etc.) Active field verification
6	Insufficient supply of MEG and medical inputs	Weakening of overall care	Advocacy and urgent supply of essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited training	High risk of nosocomial infections (96 HCW cases including 19 deaths, case fatality rate 19.7%)	Urgent supply, strengthening of IPC training
8	Stock of samples not yet analyzed (backlog) at the laboratory North Kivu and late diagnosis	Delayed confirmation, high lethality (56.3%)	Deploy a laboratory catch-up plan, strengthen diagnostic capabilities
9	Insufficient number of ambulances and isolation facilities (gap of 20 centers)	Transfer delays and isolation increase the risk of transmission	Urgent deployment of additional logistical resources and infrastructure
10	Insufficient and weak coordination use of COUSP frames	Operational inefficiency and delayed decisions	Strengthen coordination mechanisms at all levels, and make provincial COUSPs operational.
11.	Insufficient resources financial (gap of 20 million USD)	Limitation of the implementation of all pillars of the response	Urgent mobilization of resources, advocacy with partners
12	Insecurity and limited access (CODECO, ADF)	Restrictions on operations, limited access to high-risk areas	Strengthening security coordination and humanitarian access
13	Continuity of essential services (CEHS) compromised	Increase in indirect mortality (non-Ebola)	Effective implementation of free healthcare, strengthening of CEHS
14.	High population mobility	High risk of rapid geographic expansion	Strengthening surveillance at PoE/PoC sites and cross-border coordination

6. PHOTOS OF FIELD ACTIVITIES



Population Mobility Mapping (PMM) Session, Lubero-Riposte Territory, 17th
MVE with the support of the IOM

**POUR TOUTE INFORMATION
SUPPLÉMENTAIRE, VEUILLEZ CONTACTER**

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Capacity building for RECOs on Ebola Virus Disease in Bunia

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**Avec l'appui technique de l'Organisation Mondiale
de la Santé (OMS), de Africa CDC et Bluesquare**